

Click Health BRD 1.0

A simple, execution-ready brief to help engineering build a working demo of an AI-driven, closed Doctor/HCP→Patient learning platform (CO-PILOT) that delivers real, queryable insights (Click Rx Insights) to the Marketing Head (payer)

All Acronyms are mentioned on Pg 12

Assumptions

- Therapy: Type 2 Diabetes(This is only for demo. This could be any speciality- Card/Psy/Onco/Derma etc)
- Languages: English + Hindi (To start with)
- Users: Marketing Head/Product Manager, Medical Team/Product Manager (approvals), HCP (Doctor), Patient (invite-only), Sales Managers/Line Managers, Medical Representatives (MRs), Compliance/DPO (internal)
- Compliance: UCPMP 2024 and DPDP 2023 aligned
- No Rx or PHI; minimal PII (OTP); explicit consent
- CO-PILOT (Clinical Operations Platform for Insights, Learning, Outcomes, Telemetry)- Sub brand name for HCP-Patient engagement platform
- Click Rx Insights – Sub brand name for AI-powered conversational research and analytics platform to support Pharma Marketing team

Step 1: Objectives

- Marketing Campaign Management
 - Plan, run, and track HCP→Patient education campaigns by therapy, region, language.
 - View performance and ROI; export briefings for management.
- HCP Engagement (with closed Patient outreach)
 - Provide a private HCP utility to discover approved content and share via invite-only links to Patients.
 - Track de-identified HCP and Patient engagement signals.
- Content Management (Medical Team/Product Manager approvals)
 - Upload, approve (on-label only), publish, and track usage of assets (PDF, video).
- Seamless Communication
 - Share via WhatsApp/SMS (stubbed for demo), email; show delivery/use signals.
- Sales Force Enablement (for campaigns)
 - MR/Sales Manager-assisted HCP onboarding, share packs, territory snapshots, and feedback collection.
- Insights & Analytics
 - Click Rx Insights (natural-language Q&A) + PET Lite dashboards; anomaly alerts.

Step 2: Key Stakeholders and Their Workflows

1. Marketing Head/Product Manager (payer)
 - Responsibilities
 - Plan and approve campaigns; monitor performance; ask questions in natural language; export briefings.
 - Ensure activities remain ethical and on-label; coordinate with Medical Team/Product Manager for approvals.
 - Platform needs
 - Campaign setup, PET(Promotional Efforts Tracking) Lite dashboard, Click Rx Insights (NLQ), anomaly alerts, exports, compliance snapshot.
2. Medical Team/Product Manager (approvals)
 - Responsibilities
 - Approve content on-label; maintain audit logs; manage expiries/reviews.
 - If no Medical Team exists, the Product Manager acts as approver and coordinates external medical review if needed.
 - Consent governance, audit logs, DSAR support; ensure DPDP/UCPMP alignment.
 - Platform needs
 - Content upload/metadata, approval toggle (Pending→Approved), audit trail, expiry reminders.
3. Doctors (HCPs)
 - Responsibilities
 - Use approved content; invite closed Patient groups; complete micro-learning; ask on-label questions.
 - Platform needs
 - HCP portal, content library, invite-only Patient links, micro-learning and quiz, on-label CO-PILOT answers with citations.
4. Patients (Invite-only, closed group)
 - Responsibilities
 - Access HCP-shared content privately; consent to view; complete content and optional quizzes.
 - Platform needs
 - Invite link → consent → simple viewer → completion tracking (pseudonymous); no PHI.

5. Sales Managers / Line Managers

- Responsibilities
 - Oversee MR execution; ensure campaign implementation; provide field feedback.
- Platform needs
 - MR activity snapshots, campaign implementation view, simple reports.

6. Medical Representatives (MRs)

- Responsibilities
 - Onboard HCPs of respective territories; share approved packs; record campaign implementation; collect feedback.
- Platform needs
 - HCP invite tools, content packs, simple reporting.

Step 3: Key Features of the Platform (Demo MVP)

- Campaign Management
 - Create campaign (therapy, cities/territories, language, content pack, dates); track basic results.
- HCP Utility + Closed Patient Outreach
 - HCP login (OTP), consent, content library, invite-only Patient links (expiring), de-identified engagement views.
- Content Management + Approvals
 - Upload metadata; approve on-label; show “Approved v1.0” with review date; audit trail.
- Communication Tools
 - WhatsApp/SMS link sharing (stub), email updates; counters for share/use.
- Insights & Analytics
 - PET Lite: HCP WAU, HCP shares, Patient completions, top content, territory heatmap, anomalies.
 - Click Rx Insights: NLQ answers with citations to content IDs and related charts/tables.
- Compliance by Design
 - DPDP consent screens (HCP/Patient), no Rx or PHI, minimal PII, de-identified telemetry, DSAR export stub.

Step 4: Detailed Workflows

Marketing Head/Product Manager

- Login → Dashboard: HCP WAU, HCP shares, Patient completions, Top content, Territory heatmap
- Create Campaign: therapy=T2D, cities (e.g., Mumbai/Pune), language (EN/Hindi), select 5 assets, set 7-day window
- Monitor PET Lite: trend cards, heatmap, top assets, anomaly alerts
- Ask Questions: use Click Rx Insights to get answers with metrics and citations
- Export: CSV/PNG of top content, performance summary; compliance snapshot

Medical Team/Product Manager (approvals)

- Upload asset metadata → set therapy/language/tags/type
- Approve on-label content → status becomes Approved; approver + time logged
- Review expiry reminders; re-approve as needed
- If no Medical Team: Product Manager is the approver and records external medical review if applicable

HCP (Doctor)

- OTP sign-up → specialty → explicit DPDP consent
- Browse content (Diabetes, EN/Hindi) → open asset
- Create invite-only Patient link(s) → share via WhatsApp/SMS (stub)
- Track de-identified Patient views/completions
- Complete micro-learning + quiz → badge
- Ask CO-PILOT → on-label answer with citations; blocks off-label prompts

Patient (Invite-only)

- Click invite → consent → simple viewer (mobile-first)
- Complete assigned content → completion tracked pseudonymously
- Optional thumbs/quiz; no PHI; can opt-out

MR / Field (campaign enablement)

- Invite HCPs; share pack; log short visit/feedback
- Territory snapshot: which HCPs are active, which need help

Step 5: Marketing Head Demo Script + NLQ Query Catalogue

Scenario (seeded for demo)

- Cities: Mumbai, Pune
- Campaign: “Mumbai Diabetes Week”
- Assets: Foot Care (Hindi), Diet (Hindi), Diet (EN), Exercise (EN), Hypoglycemia Safety (EN)
- Seeded pattern: Hindi spikes mid-week in Andheri; underperformance in Pune

Demo Script (simple flow)

1. Dashboard opens
 - Sees HCP WAU 38%, Patient completions 124 (+22% WoW), Top content: Foot Care (Hindi)
2. Create campaign
 - T2D → Mumbai → select 5 assets → 7 days → Confirm
3. PET Lite review
 - Trend cards show Hindi completions up in Andheri; heatmap shows Borivali lagging; anomaly card appears
4. Ask key questions in Click Rx Insights (answers show bullets + metrics + citations)
 - What worked this week in Mumbai and why?
 - Which HCP cohort should we enable to improve Borivali?
 - Which 2 assets should we promote next week and where?
 - Where did we underperform vs last week and what’s the fix?
 - Compare EN vs Hindi completion rates in Mumbai; recommend next action
5. Export
 - 1-pager summary (CSV/PNG tables + bullets); compliance snapshot

NLQ Query Catalog (build these intents with filters: window, city/territory, language, therapy, content type)

Performance and “what worked”

- Top 5 assets this week and reasons
- Best-performing asset in Mumbai (by Patient completions)
- Rank assets by HCP shares vs Patient completions (7d/14d)
- Two assets to promote next week with city recommendation
- Which content themes drove highest completion (diet, exercise, safety)?

HCP engagement

- Active HCPs by city this week

- HCPs with declining activity (last 7d) for enablement
- Average dwell time by specialty and asset
- New vs returning HCPs: share contribution

Patient engagement (closed group)

- Patient completions by city and language (7d/14d)
- HCP cohorts generating most Patient completions
- EN vs Hindi completion rate comparison for selected city
- Expired invites without views

Campaign analysis

- Campaign “Mumbai Diabetes Week” performance summary
- Over/under-performing assets vs previous week
- City-wise breakdown and top contributing HCPs (de-identified rollups)

Territory and trends

- Biggest week-over-week lift by territory
- Underperforming city vs 4-week average and corrective suggestion
- Trend of Patient completion rate over last 4 weeks

Anomalies and explanations

- List anomalies detected this week
- Explain spike in Foot Care (Hindi) on Wednesday
- Any sudden drop in a key asset?

Message and language optimization

- Compare Diet vs Exercise themes (by city, by language)
- Tags correlated with higher completion rates
- Suggest 3 copy/format tweaks to improve Hindi completion

Cohorts and segmentation

- Compare Mumbai vs Pune results (top assets, completion rate)
- Which HCP cohorts (by specialty or tenure) drive better shares?
- Which patient cohort (by city) responds best to Hindi?

Action recommendations (plain English)

- Single most impactful action for next week
- Which HCP cluster to enable and with which asset pack

- Recommend 5-asset pack for Tier-2 cities

Quality and governance

- Assets with low dwell but high opens (needs revision)
- Assets approaching review/expiry
- Any off-label questions blocked this week?

Compliance snapshot (summary level)

- Confirm all answers cite approved content only
- Last 10 approvals with approver and timestamps
- Export compliance snapshot for this week

Exports and briefings

- Prepare a 1-pager weekly summary (bullets + top tables)
- Export top content table as CSV
- Generate “Do Next” check-list with links

What-if (templated)

- If we double Hindi content in Mumbai, which metrics likely improve?
- If we sunset the lowest-performing asset, expected impact?

Step 6: **Demo Script (Marketing Head deep dive)**

Setup

- Cities: Mumbai, Pune
- Campaign: “Mumbai Diabetes Week”
- Seeded assets: “Foot Care (Hindi)”, “Diet (Hindi)”, “Diet (EN)”, “Exercise (EN)”, “Hypoglycemia Safety (EN)”
- Seed events: spikes in Hindi content in Andheri mid-week; underperformance in Pune

Walkthrough

1. Open dashboard
 - Sees HCP WAU 38%, Patient completions 124 (↑22% WoW), Top content: “Foot Care (Hindi)”
2. Create campaign
 - Therapy=T2D; pick 5 assets; cities=Mumbai; window=7 days → confirm
3. Review PET Lite
 - Trend: Hindi Patient completions up; Heatmap: Andheri high, Borivali low; Anomaly card visible
4. Ask NLQ (examples)
 - What worked this week in Mumbai and why?
 - Answer: Hindi “Foot Care” and “Diet” drove 58% of completions; reason: HCP shares from Andheri cohorts; show numbers + citations [asset-12, asset-07]
 - Which HCP cohorts should we enable to improve Borivali?
 - Answer: 6 low-activity HCPs; recommend “Diet (Hindi)” pack; expected +12–18 completions; citations
 - Which 2 assets should we promote next week?
 - Answer: “Diet (Hindi)” and “Hypoglycemia Safety (EN)” based on trajectory; provide lift rationale and city targets
 - Where did we underperform vs last week and what is the fix?
 - Answer: Pune –14% completions; recommend “Exercise (EN)” localized messaging; target Kothrud; citations
 - Compare EN vs Hindi completion rates in Mumbai
 - Answer: Hindi 2.1x EN completion; suggest adding Hindi variant for “Exercise”
 - Export a 1-pager summary and a CSV of top content
 - Action: CSV/PNG generated for shareout

5. Recommendations Expected outputs in UI

- Answers with bullets, key metrics, and “Why it worked” reasoning
- Citations linking to asset IDs and charts
- A “Do Next” checklist with one-click deep links to campaign setup or HCP cohorts

Acronyms: CO-PILOT – Clinical Operations Platform for Insights, Learning, Outcomes, Telemetry; HCP – Healthcare Professional; MR – Medical Representative; MLR – Medical, Legal, Regulatory; UCPMP – Uniform Code for Pharmaceutical Marketing Practices (India, 2024); DPDP – Digital Personal Data Protection Act, 2023; ABDM – Ayushman Bharat Digital Mission; PET – Promotional Effectiveness Tracking; NLQ – Natural-Language Query; CME – Continuing Medical Education; DPO – Data Protection Officer; DSAR – Data Subject Access Request; PII – Personally Identifiable Information; PHI – Protected Health Information; HIE-CM – Health Information Exchange & Consent Manager; EMR – Electronic Medical Record; EHR – Electronic Health Record; RAG – Retrieval-Augmented Generation; NLU – Natural Language Understanding; SSO – Single Sign-On; OIDC – OpenID Connect; SCIM – System for Cross-domain Identity Management; DLP – Data Loss Prevention; SOC 2 – System and Organization Controls 2; ISO 27001 – Information Security Management Standard; WAF – Web Application Firewall; KMS – Key Management Service; SAST – Static Application Security Testing; DAST – Dynamic Application Security Testing; PWA – Progressive Web App; KPI – Key Performance Indicator; NPS – Net Promoter Score; DR – Disaster Recovery; RPO – Recovery Point Objective; RTO – Recovery Time Objective; SLO – Service Level Objective; SLA – Service Level Agreement; SDK – Software Development Kit; API – Application Programming Interface; WAU/MAU – Weekly/Monthly Active Users; CTR – Click-Through Rate; KOL – Key Opinion Leader; eRx – Electronic Prescription; MMM – Marketing Mix Modeling; BSP – Business Service Provider (WhatsApp); PV – Pharmacovigilance; ABAC – Attribute-Based Access Control; RBAC – Role-Based Access Control.